

Pain Reassessment in the Emergency Department After Initial Analgesia

(A 7-Day Retrospective Clinical Audit)

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01 Introduction

Pain is one of the most common reasons patients attend Emergency Department (ED) the Prompt assessment and management are crucial not only for patient comfort but also for ensuring quality and timely care. National guidance from the Royal College of Emergency Medicine (RCEM) 2021 recommends that initial observations should be completed within 15 minutes of patient arrival and that pain should be reassessed within one hour of administering analgesia.

02 Objective

To examine if pain reassessment is carried out within recommended timeframes after giving initial pain relief to adult patients in A&E.

- Determine the proportion of patients receiving analgesia and subsequent pain reassessment.
- Compare reassessment rates within 1 hour between those with timely (<=15 minutes) and delayed (>15 minutes) initial observations.
- Identify which staff conducted pain reassessments.
- Highlight documentation gaps around analgesia decisions.

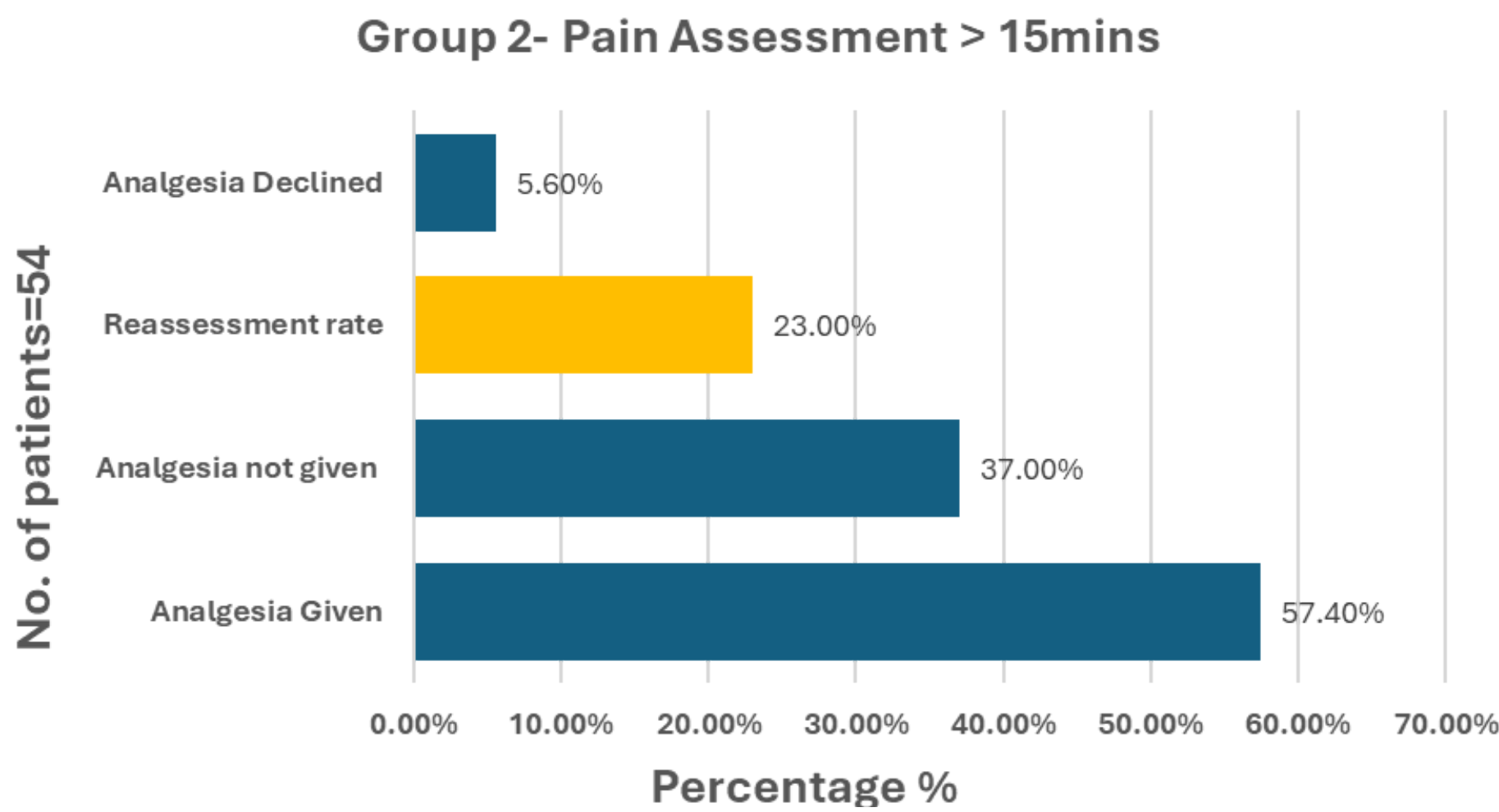
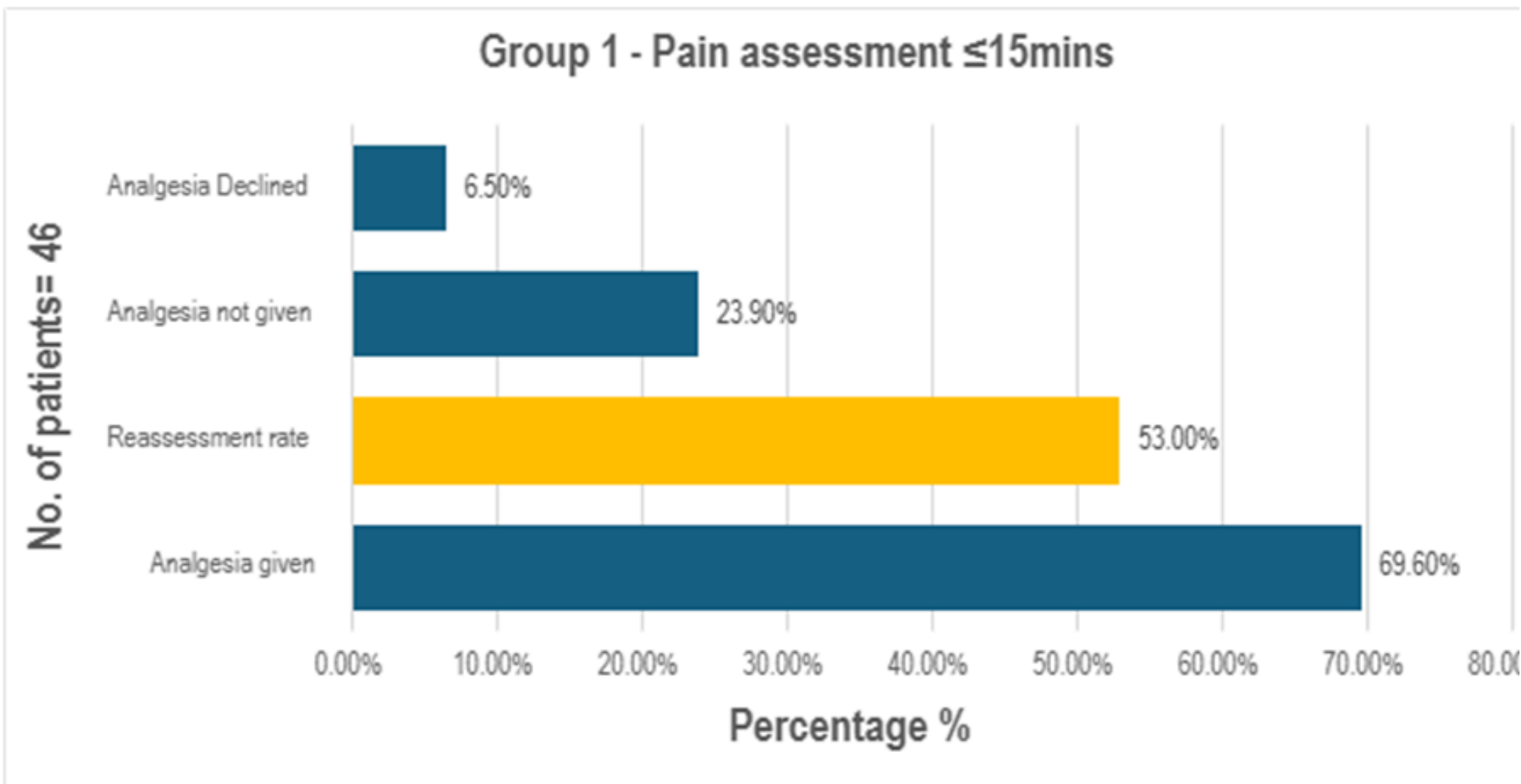
03 Methodology

A retrospective review of 100 adult patient records was conducted over a 7-day period. Patients were grouped as follows:

- Group 1: Initial pain assessment within 15 minutes (n = 46)
- Group 2: Initial pain assessment delayed beyond 15 minutes (n = 54)

04 Analysis

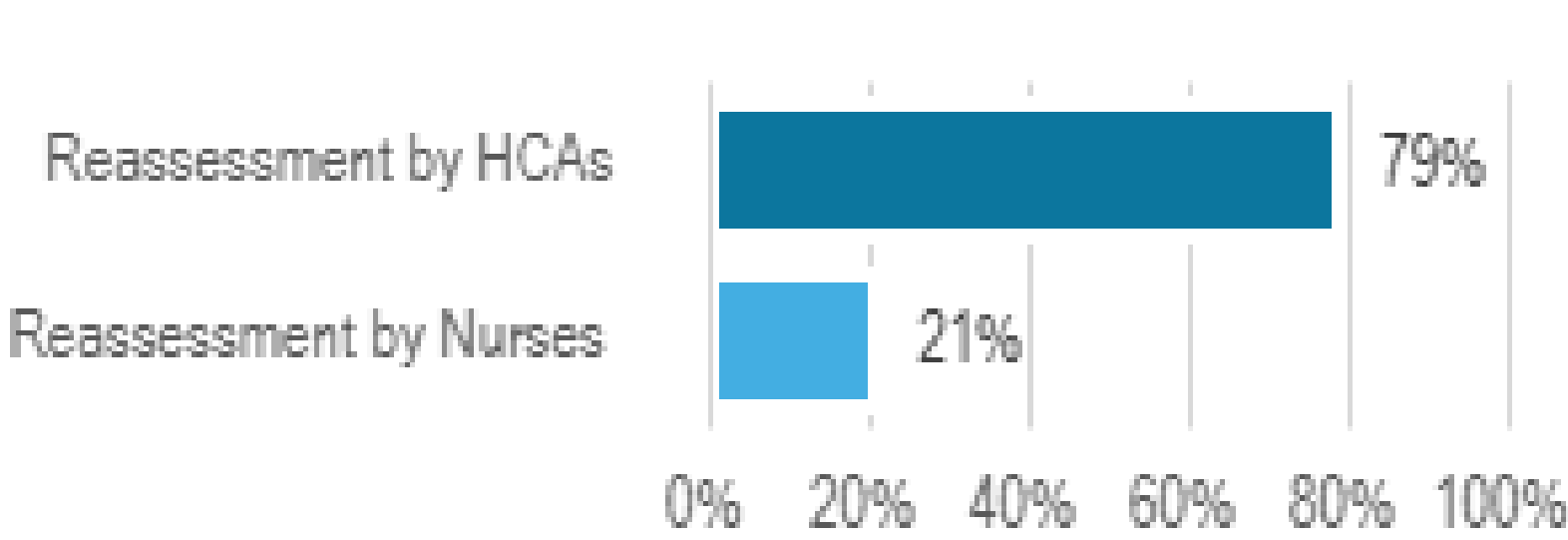
- Reassessment was more common in patients with initial pain assessment ≤15mins than >15mins(53% vs 23%).
- Only 38% of patientst given analgesia were reassessed within 1 hour.
- HCAs completed 79% of reassessments and 21% by nurses.
- Documentation was often missing on whether analgesia was declined.
- Significant delays in giving analgesia, especially in patients with late observations.
- Time to analgesia ranged from 1 minute to 7 hours 56 minutes, with longer delays in patients assessed later.



05 Results/Findings

Overall Outcomes (100)Patients):

- Analgesia Given: 63%
- No Analgesia Given: 31%
- Analgesia Declined: 6%
- Pain Reassessment (within 1 hour of analgesia): 38%



06 Conclusion & Recommendations

Patients with timely initial observations were more likely to receive pain reassessment within one hour. Overall compliance with reassessment standards was low (38%), with most reassessments performed by HCAs. Poor documentation on declined analgesia was noted. Improvements in early assessment, clear staff roles, and accurate record-keeping are needed to enhance pain management in A&E.

Staff should receive regular training on pain reassessment and documentation standards. Triage processes should be improved to ensure timely initial observations. Clearer delegation of reassessment responsibilities, especially among nursing staff, is needed. A follow-up audit is recommended to measure improvement and maintain compliance with national guidelines

Acknowledgments

ED Research Team
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Related Literature

- RCEM Best Practice Guidelines (2021)